

ChildLed AI: Clinical Assessment Protocol & Goal Template Manual for Gestalt Language Processors

1. Foundations of Spontaneous Language Sampling (SLS) Protocol

The Gold Standard of Authentic Assessment

For Gestalt Language Processors (GLPs), Spontaneous Language Sampling (SLS) is the non-negotiable "gold standard." Unlike standardized, compliance-based tools that often penalize a child for their natural processing style, SLS captures the child in their most authentic state. By shifting our focus from directive, prompt-heavy testing to child-led observation, we establish a foundation of trust and connection. This clinical shift is essential: language will not flow where connection is absent. We are not merely assessing speech; we are validating the child's unique way of being in the world.

Environmental Setup and Sensory Profiling: The "Lead SLP" Guidelines

To ensure the brain is "online" for communication, we must prioritize sensory regulation over clinical convenience. Movement is a primary facilitator of vocalization for many GLPs.

- **Sensory Integration:** The assessment space should include options for high-intensity movement (swings, climbing equipment) and sensory-rich preferences (light-up toys, tactile putty, music).
- **The "2 Old, 1 New" Rule:** To minimize performance pressure and foster intrinsic motivation, utilize a strategic toy selection protocol. Present two familiar, preferred items ("Old Items") to build comfort and one "New Item" to invite novel interaction. Avoid having a rigid play agenda; let the child's interest dictate the flow.
- **Strategic Silence:** Use silence as a tool to reduce auditory load and performance anxiety, providing "brain space" for creative praxis and initiation.

The SLS "Golden Rules" for Transcription

Accurate stage mapping requires raw, unadulterated data.

1. **Sampling Duration:** Capture a minimum of 12 minutes of active, child-led play.

2. **Utterance Volume:** Aim for at least 50 spontaneous utterances to ensure statistical validity.
3. **Verbatim Accuracy:** Transcribe exactly what is heard. This includes partner turns and "unintelligible jargon" with rich intonation.
4. **Contextual Documentation:** Note the child's body language, movement, and the environmental context of the utterance.
5. **Exclusion Criteria:** Do not include prompted language, drilled labels, or "Question/Answer" interactions. These represent compliance, not the child's natural linguistic engine.

The Parent-Clinician Collaboration

We must view parents as co-detectives. Parent interviews and home videos are vital for finding the "original source" of a gestalt. Strategic use of media search engines (e.g., *playphrase.me*, *getyarn.io*) helps us decode the emotional meaning behind a script. Understanding the original context—whether a Disney movie or a family trip to the zoo—is the key to unlocking the child's communicative intent.

Once this raw data is meticulously collected, we transition from the "detective" phase to the analytical process of NLA stage scoring.

2. NLA Scoring & Stage Analysis Engine

Beyond "Disordered" Speech: The NLA Continuum

Mapping language to the six NLA stages is a strategic clinical move that reframes the child's communication. We are no longer looking at "meaningless" scripts; we are identifying a child's location on a natural, "whole-to-part" developmental continuum. This analysis allows us to provide language supports that align with the child's natural path toward flexible, self-generated grammar.

Stage-by-Stage Scoring Rubric (Revised 2024 Guidelines)

Stage	Classification	Key Characteristics
Stage 1	Delayed Echolalia	Use of whole gestalts (intonationally defined units) to communicate.

Stage 2	Mitigated Gestalts	Breaking down gestalts into smaller chunks; mixing and matching semi-flexible units.
Stage 3	Referential Single Words	Isolation of single words; combinations of referential single words (the "Word Baby" stage).
Stage 4	Original Grammar	Pre-sentence grammar in novel phrases and early grammar in first sentences.
Stage 5-6	Advanced Syntax	Use of complex sentences and advanced self-generated grammar.

The Scoring Mechanics: Zeros and Splits

- **Taught Phrases (Score 0):** Utterances that have been drilled or taught as labels are scored as zero. These phrases are clinically "stuck" and do not provide the "raw material" for Stage 2 mitigation. Because they were not acquired naturally, they often fail to integrate into the child's developing language system.
- **Split-Scoring:** For utterances that bridge stages—such as a child using a whole gestalt while beginning to trim portions—apply split-scoring to accurately reflect this developmental shift.

Calculating the 50% Readiness Threshold

A child is only considered "ready" to move to the next developmental level when they demonstrate features of their current stage spontaneously in at least 50% of their sampled language. This threshold ensures the child has a sufficiently broad repertoire of units (gestalts or mitigations) to sustain the more complex linguistic work of the following stage.

Identifying the primary NLA stage allows the clinician to move into the selection of functional communicative models within the ChildLed framework.

3. The ChildLed Gestalt Check-In: Communicative Functions

The Strategic Mandate: Building Variety, Not Volume

In Stage 1, our goal is variety, not length. We must resist the urge to "model too much." We are looking for highly mitigatable, functional language that resonates with the child's internal experience.

The Lead SLP Warning: The Three Mistakes Clinicians must avoid these common pitfalls:

1. **Jumping In Prematurely:** We mandate a period of trust-building before a single model is offered. Models will fall flat without trust and validation.
2. **Adult-Centric Modeling:** Do not choose gestalts based on what *you* find useful. Use your detective work to find what is meaningful to the *child*.
3. **Modeling Mitigation Inadvertently:** Do not model Stage 2 or 3 chunks for a Stage 1 child. Focus on whole, mitigatable units.

Categorizing Communicative Intent

Ensure Stage 1 gestalts represent diverse functional needs:

- **Transitions:** "Time to go," "What's next?"
- **Protests/Help:** "It's stuck," "Need some help."
- **Commenting/Shared Joy:** "It's so fun," "That's so big."
- **Initiating Play:** "Let's play," "Wanna run?"

The Gestalt Selection Checklist

When selecting 2-3 potential models to use without expectation, verify the following:

- ☐ **Kid-Friendly:** Is this natural for a child to say?
- ☐ **Joint Perspective:** Does it use "Let's," "We," or "I'm"?
- ☐ **Contraction Requirement:** Does the model include a mitigatable contraction (e.g., "It's," "That's," "We're") to allow for future Stage 2 trimming?
- ☐ **Authentically Rooted:** Does it match the child's current emotional state and play interests?

Understanding the child's words is only half the battle; the rest of the clinical story is told through movement and melody.

4. Multi-Modal Detective Work: Prosody, Pitch, and Echopraxia

The "Prosody Detective" Role

For many GLPs, the melody of the speech carries the meaning. Even when vocalizations are "unintelligible," a rich intonational match to a media clip or a family script confirms the existence of a Stage 1 gestalt.

The Diagnostic Log: Jargon and Media

Clinicians should log instances of jargon and "vocal play."

- **Prosody Match:** Note if the jargon has a "rich intonation" matching a specific video or song.
- **Diagnostic Markers:** If a child "studies and replays media clips" or shows no progress on a traditional AAC layout despite intensive intervention, these are strong diagnostic indicators of a GLP.

Echopraxia (Movement Gestalts)

Echopraxia is the voluntary use of movement to convey a gestalt.

- **Definition:** Communicating a larger experience (e.g., acting out a full YouTube scene or replicating a character's facial expression) to convey an emotion. It is *not* involuntary mimicking.
- **Interdisciplinary Directive:** Educate the team (teachers, OTs) that Echopraxia is a valid, functional communication act. Do not allow it to be labeled as "non-functional" or "behavioral."

Interpreting Receptive Results and Regulation

Standardized receptive tests fail GLPs for two primary reasons:

1. **The Regulation Factor:** A child's true abilities are only visible when they are regulated. If dysregulated, echolalia may be a self-regulation tool, not social communication.
2. **Linguistic Hierarchy:** Per NLA research, GLPs do not recognize single words as units of meaning until **Stage 3**(Referential Single Words). Furthermore, they cannot flexibly answer "WH" questions until **Stage 4** (Self-generated language). Standardized vocabulary pointing tasks and question-answering assessments are developmentally inappropriate for children in Stages 1 or 2.

This comprehensive analysis provides the evidence base for formulating neurodiversity-affirming goals.

5. Neurodiversity-Affirming Goal Template Bank

Shift to Connection-Based Objectives

All goals must be written from the child's or joint perspective. We avoid "you" models or expansion techniques (e.g., "Child will add one word to noun") because expansion is an analytic processing strategy that does not apply to the gestalt developmental path.

Stage 1-3: Connection and Mitigation

- **Stage 1:** "Child will increase the variety of functional, mitigatable gestalts (e.g., transitions, protests, shared joy) by acquiring [Number] new spontaneous gestalts across communicative functions during child-led play."
- **Stage 2:** "Child will demonstrate mitigation by 'trimming down' or recombining chunks from two established gestalts (e.g., 'Let's play' + 'outside') in [Number] spontaneous interactions."
- **Stage 3:** "Child will isolate referential single words and use combinations of referential single words (noun + noun) during child-led interactions in [Number] opportunities."

Stage 4-6: Original Grammar

- **Stage 4+:** "Child will use self-generated grammar in novel phrases and first sentences, as tracked by Developmental Sentence Types (DST), during spontaneous conversation."

The Mandatory Support Clause

To ensure every goal is neurodiversity-affirming, clinicians must append the following: *"...with access to preferred sensory movement, tools, and a child-led environment that presumes competence."*

6. Clinical Documentation: SOAP & Progress Note Exemplars

Telling the Clinical Story

Documentation must prioritize the clinician's detective work and the child's engagement. We do not use "correct/incorrect" percentages for gestalt acquisition.

The GLP SOAP Note Framework

- **Subjective:** Log regulation and sensory needs (e.g., "Child required 10 minutes of heavy work/climbing to reach a regulated state").
- **Objective:** List new gestalts, specific mitigations, or movement gestalts (Echopraxia) observed.
- **Assessment:** Interpret today's data against the 50% readiness threshold for the next NLA stage.
- **Plan:** Identify the next 2-3 potential gestalts to model naturally without expectation.

Progress Note Templates

Template A: Stage 1 (Variety Focus)

- **Primary NLA Stage:** 1
- **Progress Summary:** (Name) is communicating via Stage 1 gestalts. We observed an increase in functional variety, specifically within the "Transition" category (e.g., "Time to go"). We continue to perform detective work on unintelligible jargon with rich prosody to decode media-based scripts.
- **Support Clause:** (Name) was supported by a child-led environment with access to [Sensory Tool].

Template B: Stage 4 (Grammar Emergence)

- **Primary NLA Stage:** 4
- **Progress Summary:** (Name) has surpassed the 50% threshold for Stage 3 and is now using original, self-generated grammar. We are seeing novel phrases and first sentences that reflect the child's original thoughts rather than scripts.
- **Support Clause:** (Name) remains most communicative during high-movement play (e.g., swinging).

The clinician's ultimate role is that of a validator. We are play partners who honor the child's unique linguistic journey from the "whole" to the "part."